ACTT
MENTAL HEALTH EMERGENCY

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Possible MH Emergencies

- Agitation/Psychosis/Delirium
- Suicidality

Objectives

- List broad differential diagnosis for an agitated patient
- Outline management options for agitation
- Review red flags on history for suicidality
- Review approach to managing patients experiencing suicidal ideation

Case 1

- You are the PA sailing on HMCS Ottawa. The coxswain and one of the boatswains bring in 23-year-old S1 Hulk to the Sick Bay with concerns that the patient is “behaving erratically.”
- Apparently he threw his plate against the wall in the galley— “He’s not acting himself” “Yelling and being physically violent”

What is your differential for an agitated patient?

Differential diagnosis for agitation

- Toxicology
  - Sympathomimetics (cocaine, meth)
  - Marijuana
  - Anticholinergics
  - Serotonin syndrome, NMS
  - Etc.
- Metabolic/Endocrine
  - Hypoglycemia
  - Hypoxia
  - Thyrotoxicosis
- Structural CNS
  - Trauma
  - Stroke, mass, lesions
- Infection
  - Sepsis
  - Meningitis
- Primary psychiatric disorder
  - Schizophrenia, Schizoaffective disorder, Bipolar, Etc.

Case 1 continued

- You try to engage in a conversation but S1 Hulk tells you to “F*** off”
- Med Tech tries to get vitals but S1 Hulk puts his hands up and says “Don’t touch me or I swear I’m gonna hurt you!”

What are your next steps?

Levels of agitation

- Mild
  - Agitated but cooperative
- Moderate
  - Disruptive without danger
- Severe
  - Excited delirium and/or dangerous to self/others
  - **Medical emergency

What level of agitation is our patient demonstrating?

Management of agitation

- Mild-moderate:
  - Attempt VERBAL DE-ESCALATION
- Moderate-severe:
  - Always attempt verbal de-escalation first if possible
  - PHYSICAL RESTRAINT
  - CHEMICAL RESTRAINT

Verbal De-escalation

What are some verbal de-escalation techniques that you can use?

Verbal De-escalation

- Ensure **safety** (self/others)***
- **Non-verbal** cues:
  - Quiet, calm environment (one speaker only)
  - Hands visible, relaxed posture
- **Verbal** cues:
  - Calm, low, slow voice
  - Short, simple sentences
- **Validate**:
  - Ex/ “I can see you’re really upset” “I can only imagine what you’re going through right now”
- Set **clear limits**:
  - Ex/ “I want to talk to you, but I can’t do that if you’re threatening staff”
- Offer **choices** (restore control):
  - Ex/ “Would you rather sit here or move to a quieter room?”
  - Ex/ “Can I offer you something that will help you feel more calm?”

Case Progression

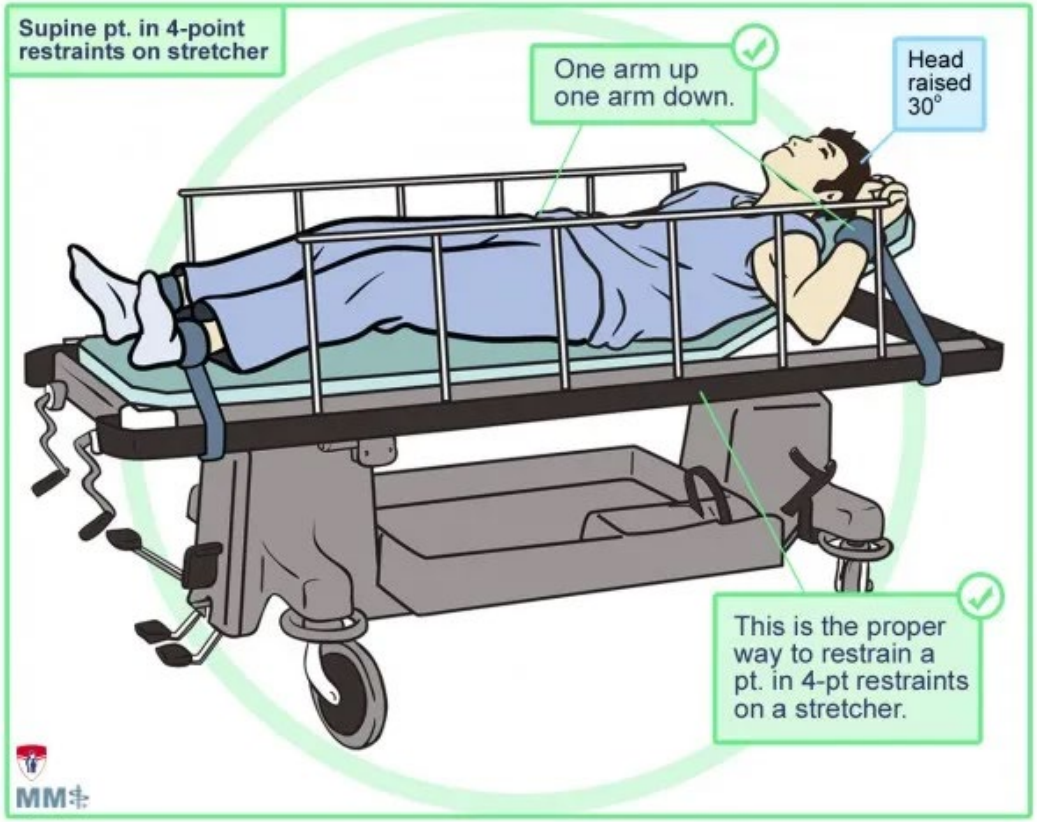
- You ensure everyone's safety by ensuring that there are no dangerous weapons nearby.
- You ask the Coxswain and the Bosun to remain outside the Sick Bay and you go in alone but stand near the door so you can quickly exit if need be.
- You try to verbally de-escalate S1 Hulk and offer him some oral sedation. To your surprise, he starts yelling and swinging his fists.

What should you do next?

Physical Restraint

- Do NOT:
  - Prone positioning
  - Apply pressure on their neck or chest
  - Restrain for prolonged periods
  - Have multiple people shouting or giving commands

Physical Restraint



What are some potential dangers to physical restraints?

EM Cases

Physical Restraint

- **LAST RESORT
  - Used only as a *bridge* to chemical sedation
- Ideally, 6 trained staff (one for each extremity, one for head, one for administering the medications)
- Carries HIGH risks:
  - Asphyxia & respiratory compromise
  - Cardiovascular collapse
  - Hyperthermia
  - Rhabdomyolysis
  - Traumatic injuries (both patient and staff)

How about chemical sedation? What would you use?

Chemical Restraint

PADIS	Routes	IV Dose	Peak Onset	Half Life
Lorazepam	PO/IV/IM	1-2 mg	**15-20 mins**	8-12 hrs
★ ***Midazolam***	PO/IN/IM/IM	**5-10 mg**	5-8 mins	**2** hrs
★ ***Diazepam***	PO/IV/pR Not recommended to give it IM	**5-10 mg**	5-8 mins	**24** hrs
Haloperidol	PO/IV/IM	5 mg	20-25 mins	24 hrs
Ketamine	PO/IV/IM	**IM Dose 4 mg/kg**	10-15 mins	2 hrs

Midazolam 10mg IM

Case Progression

- With the help of six sailors, you are able to safely hold S1 Hulk down as you successfully administer 5mg of IM Midazolam into his lateral thigh.
- After 5 minutes, S1 Hulk start to calm down and drift off to sleep.

What are your next steps?

Severe agitation (Excited Delirium) is a MEDICAL EMERGENCY

- Treat these case like any other life-threatening medical cases
  - Transfer to resus bay
  - MOVIES
  - ABCDE
- Treat your vital signs
  - Hypoxia —> O2 (advanced airway?)
  - Hyperthermia —> Cooling
  - Hypertension & Tachycardia —> More sedation
- Look for underlying cause (DDx)

Case End

- With the help of your team, you safely transfer S1 Hulk to the patient bed. You manage to get initial vitals (BP 160/100, HR 120, RR 28, SpO2 100% RA, T 38.4), set up monitors and establish an IV.
- Snoring but protecting airway
- Pupils dilated, sweaty. Cooled with ice packs.
- No signs of trauma
- No clonus or muscle rigidity
- ECG sinus tachy
- Glucose is 6.5

- You monitor his breathing with IV midazolam ready to give PRN.
- After about 2 hours, patient wakes up calm and alert. He admits to using cocaine for the first time. You let his CoC know and he is repatriated during next port visit.

Questions??

Case 2

- The next morning, SLt(N) Bloggins presents to your Sick Bay asking to speak privately.
- He tells you he is having “dark thoughts.”
- “I’m severely depressed… I look at the ocean and I can’t stop thinking about jumping off.”

How do you assess suicide risk?

Suicide Risk

Ask questions that are bolded and underlined.	Past month	
Ask Questions 1 and 2	**YES**	**NO**
1) ***Have you wished you were dead or wished you could go to sleep and not wake up?***		
2) ***Have you actually had any thoughts of killing yourself?***		
If YES to 2, ask questions 3, 4, 5, and 6. If NO to 2, go directly to question 6.		
3) ***Have you been thinking about how you might do this?*** E.g. "*I thought about taking an overdose but I never made a specific plan as to when where or how I would actually do it....and I would never go through with it.*"		
4) ***Have you had these thoughts and had some intention of acting on them?*** As opposed to "*I have the thoughts but I definitely will not do anything about them.*"		
5) ***Have you started to work out or worked out the details of how to kill yourself? Did you intend to carry out this plan?***		
6) ***Have you ever done anything, started to do anything, or prepared to do anything to end your life?*** Examples: Collected pills, obtained a gun, gave away valuables, wrote a will or suicide note, took out pills but didn't swallow any, held a gun but changed your mind or it was grabbed from your hand, went to the roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc.	**Lifetime**	
	Past 3 Months	
If YES, ask: ***Was this within the past three months?***		

Item 1 Behavioral Health Referral at Discharge
Item 2 Behavioral Health Referral at Discharge
Item 3 Behavioral Health Referral at Discharge
Item 4 Immediate Notification of Physician and/or Behavioral Health and Patient Safety Precautions
Item 5 Immediate Notification of Physician and/or Behavioral Health and Patient Safety Precautions
Item 6 Over 3 months ago: Behavioral Health Referral at Discharge
Item 6 3 months ago or less: Immediate Notification of Physician and/or Behavioral Health and Patient Safety Precautions

Suicide Risk

- Triggers?
- Active plan?
- Patient risk factors?
- Environmental risk factors?
- Protective factors?

Case Progression

- Triggers
  - Recently divorced
- Patient risk factors
  - Male
  - Hx of EtOH abuse in the past
  - Hx of childhood trauma
  - Uncle killed himself when he was 10 years old
- Environmental risk factors:
  - Deployed, away from home
  - NWO with access to ship's firearms
  - Surrounded by ocean…
- Protective Factors
  - Daughter at home
  - Strong camaraderie with fellow NWOs

- You are worried about this patient's high suicide risk based on your assessment. *What are your options for managing this patient on ship?*

Management of Suicidality

- Notify your superiors (e.g. Fleet Surgeon)
  - Immediate return to shore with escort?
  - Hospitalisation?
- Mobilize local resources (with patient consent)
  - Padres
  - Sentinels
  - Fellow comrades
- Buddy system / Suicide watch
- Schedule frequent appointments

Questions??

Key points

- Treat severe agitation as a medical emergency
- Always attempt verbal de-escalation first, if possible
- Physical restraints are TEMPORARY bridges to chemical sedation
- Develop approach to assess suicidal risk
- Have a low threshold to contact your superiors to coordinate repatriation for patients with suicidal risk